

Beyond Survival: Integrating Rehabilitation for Health, Longevity, and Equity

A high-level dialogue on rehabilitation within Universal Health Coverage, anchored in WHA Resolution 76.6 and convened during the 79th World Health Assembly.

21 May 2026 • WHO Headquarters, Geneva

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Governments of Cameroon, Japan and the Slovak Republic

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Humanity & Inclusion / Handicap International Federation • International Association of Communication Sciences and Disorders (IALP) • World Rehabilitation Alliance

Moderated by **Ms. Kamini Gadhok, MBE**, President-Elect of IALP, the dialogue was framed around three figures: 2.6 billion people globally could benefit from rehabilitation; over half of those in low- and middle-income countries do not receive it; and pressures are rising due to ageing populations, noncommunicable diseases and humanitarian emergencies.

The question is no longer whether rehabilitation belongs in health systems, but how to operationalise it.

2.6_B

people globally who could benefit from rehabilitation

>50%

in LMICs who do not receive needed rehabilitation

75+

countries now committed to the Rehabilitation 2030 Agenda

COUNTRY EXPERIENCES

H.E. Manaouda Malachie

Minister of Public Health of Cameroon

Rehabilitation services in Cameroon have historically been limited to specialised hospitals. Progress is being made integrating rehabilitation into primary health care. The way forward centres on three pillars: partnerships, training, and alignment with international frameworks — together with stronger data collection and community-based rehabilitation. Sustained progress on rehabilitation requires three things together: partnerships, workforce training, and alignment with international frameworks— supported by stronger data and community-based services.



Dr. Masato Izutsu

Senior Coordinator for Global Health, Ministry of Health, Labour and Welfare of Japan

Japan achieved universal health coverage in 1961 and has since become a global model for healthy ageing. Rehabilitation is covered under both health insurance and long-term care insurance, enabling continuity across the care continuum, with disease-specific programmes of defined duration to ensure quality. Key asks of WHO and global partners: position rehabilitation as a core UHC component, support data use and cooperation, and build strong workforce frameworks before integration.

Ms. Kristina Králiková

Director General, Section of Inpatient and Day Health Care, Ministry of Health of the Slovak Republic

Rehabilitation is already included in Slovakia's health coverage— current work focuses on optimisation. Slovakia is developing a patient classification system based on complexity to inform a new payment mechanism, drawing on practice from Australia, New Zealand, Switzerland and Austria. The goal is to link health care providers with rehabilitation providers through financial incentives and case-mix classification. Palliative care is embedded within Slovakia's rehabilitation model.

Beyond Survival — continued

GLOBAL PROGRESS ON REHABILITATION

Dr. Guy Fones

Unit Head, Department of Noncommunicable Diseases and Mental Health, World Health Organization

A **monitoring framework with agreed indicators has been finalised** and is heading to the World Health Assembly — a historic first. More than 75 countries are committed to the Rehabilitation 2030 agenda, with WHO tools — including STARS (the Systematic Assessment of Rehabilitation Situation) and GROWE (the Guide for Rehabilitation Workforce Evaluation) — in active implementation across diverse health systems. Rehabilitation is appearing in condition-specific resolutions including stroke, signalling a shift from siloed to integrated approaches. The African regional strategy sets an ambitious target: 60% of countries with a national rehabilitation plan and dedicated budget.

Three asks: explicitly include rehabilitation in UHC reforms and benefit packages; track integration through measurement; treat rehabilitation as a defining test of whether UHC is truly universal.

COUNTRY IMPLEMENTATION IN PRACTICE

Dr. Abgogbe Romaric Maurille Tognon

Rehabilitation Project Manager, Humanity & Inclusion / Handicap International Federation, Benin

Since 2019, the WHO STARS assessment has provided the evidence base for national planning, leading to the 2023–2027 National Rehabilitation Plan. Implementation is a tripartite partnership between the Ministry of Health, WHO, and civil society organisations including Humanity & Inclusion / Handicap International Federation and the ICRC. Progress spans all six strategic priorities: governance, service quality, assistive technology, workforce development, data integration into the national health information system, and financial access through a dedicated fund for vulnerable people. Genuine government–WHO–civil society partnership is the prerequisite for sustained progress.

CLOSING CALL TO ACTION: FUNCTIONING AS A THIRD DIMENSION OF HEALTH

Ms. Samantha Shann

Vice-Chair, World Rehabilitation Alliance Steering Committee

Health systems are currently judged on mortality and morbidity, but these no longer tell the full story. **Functioning — whether people can move, communicate, work and participate — must become the third measure of health.** If functioning is not measured, rehabilitation remains undervalued; measurement drives accountability, and accountability drives investment. The economic case is strong: functional decline increases long-term care costs and productivity losses; early rehabilitation generates substantial lifetime savings. The World Rehabilitation Alliance is advocating for rehabilitation to be embedded in the 2027 UN High-Level Meeting on UHC and post-2030 global health agendas.

“Health systems should not be judged on whether people survive disease, but by whether people are able to live meaningful, functional lives.”

Samantha Shann · Closing remarks

